| Quality Appraisal Checklist | Personal Characteristics | | | | Relationship with Participants | | Theoretical Framework | Participant Selection | | | | Setting | | | Data Collection | | | | |
|---|---|---|---|---|---|---|---|---|---|---|---|---|---|---|---|---|---|---|---|
| Title | Interviewer/Facilitator | Credentials | Occupation | Experience and training | Relationship established | Participant knowledge of the interviewer/interview | Interviewer characteristics | Methodological orientation and theory | Sampling | Method of approach | Sample size | Non-participation | Setting of data selection | Presence of non-participants | Description of sample | Interview guide | Repeat interviews | Audio/visual recording | Field notes | Duration | Data saturation | Transcripts returned |
| Self-monitoring lifestyle behaviour in overweight and obese pregnant women: qualitative findings | First author | Author 1: DNSc, MPH, RNC-OB, FAAN. Author 2: PhD, RN, FAAN | Author 1: Associate Professor. Author 2: Researcher | First author: Expert in prenatal health promotion and content analysis. Second author: PhD. Doctoral student carrying out analysis: undergraduate degree in exercise science, experience in a home visitation programme for low-income pregnant women | First author called or e-mailed each woman 3 - 7 days before a scheduled interview | Not mentioned - assumed through recruitment | No bias reported | Self-monitoring theory, behavioural and lifestyle change theories | Purposive | Interview explained to trial participants and informed consent sought | 13 | 22 women enrolled in the intervention, 9 refused interview | 10 interviewed in clinic private rooms, 3 over phone | No | Mean age 29.5 years, majority obese, majority were African American (77%), unmarried (69%), previous pregnancy (77%), third trimester (69%) | Yes | No | Audio | No | 30 minutes | Not discussed | No |
| "I was tired of all the sticking and poking": Identifying Barriers to Diabetes Self-Care Among Low-Income Pregnant Women | Author 1 and 2 | Author 1: MD, MPH, Author 2: MD, MPH, Author 3: MPH, Author 4: APN, CDE, Author 5: MD, MPH | Author 1: Fellow in Maternal-Fetal Medicine. Author 2: Resident in OBGYN at Northwestern. Author 3: Senior Research Coordinator at Northwestern. Author 4: Advance Practice Nurse in Maternal-Fetal Medicine. Author 5: Professor of OBGYN at Northwestern | Extensive expertise in maternal-fetal medicine and research | Not mentioned | Were identified because they had GDM so knew why the research was being conducted | No bias reported | Cognitive load theory | Purposive | At a clinic serving public-assisted low-income women. Informed consent sought. | 10 | 46 women identified, 22 ineligible, 12 declined participation | Conducted in private | No | Mean age 28.5, African American (4), Latina (4), Native American (1), Asian (1). Most educated to high school degree level, most multiparous, all diabetic | Yes | Three interviews per person | Audio | No | 30-60 minutes | Reached at 29 interviews | No |
| The maternal experience of having diabetes in pregnancy | Not reported by coder/analyser was present for all interviews and focus groups | Author 1: MD, Author 2: PhD, RN, PMHCNS-BC, Author 3: PhD, RN, IBCLC | Not reported | Extensive expertise judging by credentials | Not mentioned | Recruitment flyers provided some knowledge of subject matter | No bias reported | Phenomenology | Convenience | Flyers distributed in healthcare provider offices | 8 | Not reported | Over the phone or in person | Yes, additional investigator to take field notes | Average age 35, mostly highly educated, no demographics reported | Yes | No - these were focus groups | Audio | Yes | Not reported | Not discussed | Some were to check participants views were represented correctly |
| Women's experiences of gestational diabetes self-management: a qualitative study | Not reported | Author 1: PhD | Associate Professor Midwifery | Not reported but has a PhD in a related field | Approached by researcher prior to interview to confirm interest | Recruited from diabetes clinic fitting criteria | No bias reported | Phenomenology | Purposive (blinded for review) | Women invited to participate if fitted criteria, approached by researcher if Diabetes Coordinator confirmed their interest | 15 | Not reported | 10 phone interviews, one in interviewee's home, one focus group at the hospital clinic | No | Variety of ethnic backgrounds, 23 - 40, majority high school educated, majority primiparous | Not reported | No | Audio | No | Not reported | Not discussed | No |
| Factors promoting diabetes self-care among low-income, minority pregnant women | Author 1 and 2 | Author 1: MD, MPH, Author 2: MD, MPH, Author 3: MPH, Author 4: APN, CDE, Author 5: MD, MPH | Author 1: Fellow in Maternal-Fetal Medicine. Author 2: Resident in OBGYN at Northwestern. Author 3: Senior Research Coordinator at Northwestern. Author 4: Advance Practice Nurse in Maternal-Fetal Medicine. Author 5: Professor of OBGYN at Northwestern | Members of the research team reported to have prior qualitative research experience and extensive interviewing training | Not mentioned | Were identified because they had GDM and at their first clinic visit so knew why the research was being conducted | No bias reported | Cognitive load theory | Purposive | Recruited at their first clinic visit for diabetes care | 10 | 46 women identified, 22 ineligible, 12 declined participation | Conducted in private | No | Mean age 28.5, African American (4), Latina (4), Native American (1), Asian (1). Most educated to high school degree level, most multiparous, all diabetic | Yes, provided in article | Three interviews per person | Audio | No | 30-60 minutes | Reached at 29 interviews | No |
| Socio-cultural aspects of self-management in gestational diabetes | Author 1 | Author 1: PhD student. Author 2: Assistant Professor | Author 1: PhD student. Author 2: Assistant Professor Nursing | PhD student | Not mentioned - secondary analysis | Not mentioned - secondary analysis | Not mentioned - secondary analysis | Phenomenology | Purposive | Women being treated for GD were recruited | 12 | Not mentioned - secondary analysis | Not mentioned - secondary analysis | Not mentioned - secondary analysis | Aged between 23 - 38, most worked outside the home, most were married, mix of first pregnancy and second pregnancies | Yes | Two interviews conducted per person | Audio | Not mentioned - secondary analysis | Not mentioned - secondary analysis | Not mentioned - secondary analysis | Not mentioned - secondary analysis |
| Women's experiences of factors that facilitate or inhibit gestational diabetes self-management | Author 1 | Not reported | Not reported | Not reported | Access to women facilitated by diabetes co-ordinator, no relationship pre-established with researcher | Recruited through diabetes clinic and by diabetes co-ordinator so would have been told what the study was about | No bias reported | Interpretative phenomenology | Purposive | Diabetes coordinator approached women, participants were recruited after they attended GDM education and had at least 3 weeks self management of condition | 15 | 10 women met inclusion criteria, 20 agreed to participate, 15 available on day of interview | Focus group in private room in clinic, phone interviews, face to face interview at one woman's home | No | Age range 23 - 40, education level lower than population average, mix of ethnic backgrounds, majority of women primiparous | Not reported | No | Audio | No | Not reported | Not mentioned | No |
| Lived experience of blood glucose self-monitoring among pregnant women with gestational diabetes mellitus: a phenomenological research | Not reported | Author 1: PhD, RN, Assistant Professor of Nursing. Author 2: PhD, RN, Associate Professor of Nursing | Author 1: Assistant Professor. Author 2: Associate Professor | Not reported | Prolonged engagement was carried out by researchers to build a rapport with participants before interviews | Participants approached by staff nurses so knew about what the study was about prior to taking part | No bias reported | Phenomenology | Purposive | Approached at antenatal care clinics and informed consent obtained | 30 | Not reported | Interviewed at the antenatal clinic, diabetes clinic or obstetric ward | No | The mean age of participants 32.5 years, 17 women were Buddhists, 13 women were Muslims. Majority multiparous and employed. | Yes | Two interviews conducted per person | Audio | No | 30 - 45 minutes | Data were collected until saturation (after six months) | No |
| Living with gestational diabetes in a rural community | Not reported | Author 1: DNP, APRN-C. Author 2: DNS, WHNP-C | Author 1: Visiting Assistant Professor of Nursing. Author 2: Visiting Assistant Professor | Not reported | Established through recruitment | Had diagnosis of the disease which was being explored in interviews, and recruited via study posters | No bias reported | Phenomenology | Purposive and snowball | Information about study posted in obstetric and health care provider officers in New York State | 10 | Not reported | Interviewed in person or at phone | No | All women Caucasian between 25-49. All had history of GDM. | Yes | Follow up interviews conducted with four participants | Audio | No | 15 - 30 minutes, second interviews 15 - 30 minutes | Checked by sharing some themes with participants | Transcripts of first interview provided to participants doing a second interview for verification |
| "Is gestational diabetes a severe illness?" Exploring beliefs and self-care behaviour among women with gestational diabetes living in a rural area of the south east of China | Author 1 | Author 1: M, Msc. Author 2: RN, PhD. Author 3: RN, PhD | Author 1 a teacher of maternal care and bilingual | Not reported | Not mentioned | Interviewer did not know participants prior to study, was not part of the clinical care team. | No bias reported | No specific theoretical framework reported | Purposive | Women purposefully recruited from obstetric clinics and wards at a specific hospital. | 17 | Not reported | Interviewed in a room at the hospital | No | 27.5 median age, six women high educational level, five middle educational level, six low educational level | Yes and provided in paper | No | Audio | No | 40 - 60 minutes | Not mentioned | No |
| Experiences, perceptions and self management of gestational diabetes in a group of overweight multiparous women | Not reported | Not reported | Not reported | Interviews conducted by experts in anthropology and social psychology | Established through recruitment and screening process | Women were given a health questionnaire prior to recruitment | No bias reported | Phenomenology | Purposive | Women recruited through department of diabetes, given questionnaire and selected based on the questionnaire results | 5 | Not reported | Not reported | No | Reported in table | Yes | No | Audio | Yes | 90 minutes | Not mentioned | No |
| Ethnic Differences in Dietary Management of Gestational Diabetes Mellitus: A Mixed Methods Study Comparing Ethnic Chinese Immigrants and Australian Women | Not reported | Author 1: MN, APD Author 2: PhD Author 3: MD Author 4: PhD | Not reported | Not reported | No | Were identified because they had GDM so knew why the research was being conducted | No bias reported | Symbolic interactionist framework and grounded theory | Purposive | Women recruited through GDM clinics | 83 | 11 declined to participate and reasons were given | Not reported but some on the phone (others in person) | No | Reported in table | Yes | No | Audio | Yes | 45 - 90 minutes | Not mentioned but large sample size so assumed there was saturation | No |
| Self-management of gestational diabetes among Chinese migrants: A qualitative study | Chief Investigator | Not reported | Faculties reported | CI was fluent in English, Mandarin and Cantonese | Participant's had interviewer's contact details | Interested participants were provided with written information that explained the background of the study and provided the contact details of the researchers. All participants were recruited after they had received diabetes education from a diabetes educator and had at least two weeks of self-management experience. | No bias reported | Thematic analysis | Purposive | Between August and October 2016, participants were purposively recruited from the antenatal clinic at the RPAH in the Inner West Region of Sydney, Australia. Bilingual flyers about the study were distributed at the clinic. Interested participants were provided with written information that explained the background of the study and provided the contact details of the researchers. | 18 | Not reported | The interviews were conducted by the chief investigator in either a staff meeting room or a waiting room at the hospital, according to the participant's preference. | Yes, participant's partner present in two interviews | Reported in table: The mean age of participants was 34 years (SD = 3.6; range 29–41). The majority of participants came from China, Hong Kong or Taiwan and held a university degree | Yes | No | Audio | No | Not reported | Not reported | No |
| The experience of gestational diabetes for indigenous Māori women living in rural New Zealand: qualitative research informing the development of decolonising interventions | Not reported | Not reported | Faculties reported | Not reported | Lead researcher spoke to participants ahead of the interviewer | The contact details of all consenting participants were then provided to the lead researcher, who contacted participants via phone or home visits (when no phone service was available). The lead investigator provided detailed information about the research, answered any questions from women and obtained written informed consent and confidentiality agreements (including accessing their diabetes-related health data from the clinic) for each participant. | No bias reported | Kaupapa Maoris Methodology | Purposive - a sub sample of a larger study | The ten participants advised that they either had a clinical history of GDM (n = 8) or had been exposed to diabetes in utero (n= 2). Eligible participants were recruited via the local primary care clinic between September and December of 2015. | 10 | Reported in the larger study (6 withdrew) | Interviews undertaken in participants' homes or the health centre, or a mutually agreed venue. | No | 10 Māori women aged between 30 and 69 years. Outlined in full table, participants had diverse pregnancy experiences and current diabetes-related health conditions | Yes | No | Audio | No | Not reported | Not discussed | Participants were invited to review, edit or change their transcript prior to data analysis. |
| "Diabetes Just Tends to Take Over Everything": Experiences of Support and Barriers to Diabetes Management for Pregnancy in Women With Type 1 Diabetes | Not reported | Not reported | Not reported | The main interviewer had a background in health psychology and skills in motivational interviewing. | Potential participants could indicate interest by contacting the investigator. Study procedures and the consent process reviewed with them. | Potential participants were informed that the purpose of the study was to identify barriers to and supports for blood glucose management around pregnancy | No bias reported | Thematic analysis | Purposive | Participants were recruited using advertisement flyers that were sent for distribution to diabetes specific clinics around the United States. Adult U.S. women (≥18 years of age) with type 1 diabetes who were fluent in English and were either pregnant, planning pregnancy for the immediate future, or had experienced a previous pregnancy were invited to participate in a phone interview. | 15 | 27 indicated interest | Decided to conduct the interviews over the phone unless participants were local to the study site and wished to come to the study office | No | All participants were married, with a mean age of 34 years and a mean duration of type 1 diabetes of 21 years. Ten participants used a continuous glucose monitoring (CGM) system, and 15 used an insulin pump to manage their diabetes. Ten women had previously been pregnant, three were pregnant at the time of the interview, and five were trying for or contemplating pregnancy in the immediate future (within 0–4 months from the time of their interview). | Yes | No | Audio | Yes - notes and reflections recorded by interviewer | Between 30 and 50 minutes | To achieve data saturation, 10–15 people were recruited, with the possibility of conducting additional interviews, if necessary | No |
| Barriers to Gestational Diabetes Management and Preferred Interventions for Women With Gestational Diabetes in Singapore: Mixed Methods Study | First and second author | Author 1, PhD, Author 2 MPH, Author 3 MPH, Author 4 MBBS, MD, Author 5, MBBS, Author 6, PhD | Faculties reported | Not reported | Established through recruitment | Recruited from diabetes clinic fitting criteria, participants had previously participated in a survey on the same subject. | No bias reported | Thematic analysis | Purposive | Potential participants for the survey (this is a mixed methods study and interview participants were recruited from survey participants) were approached in the waiting room of the GDM clinic. The study team refrained from approaching women who seemed tired or anxious. After getting informed consent, women were given a self-administered survey. For pregnant women who were willing to participate in prenatal interviews, sessions were scheduled based on their convenience. In the second phase, to increase survey completion, the women were given an incentive of $25 (US $17.5). In the first phase, 50 patients participated in the survey. | 15 | Of the 50 patients who completed the survey, 35 gave verbal consent to participate in the interviews and 15 participated in the interviews | Most interviews were conducted at the health care facility or at the participants' homes, in a private space conducive for the participant to share thoughts effectively. | No | The women who participated in interviews had a demographic profile similar to that of the survey questionnaire participants. Half of the 15 participants in the age group of 31 to 35 years were primiparous or had a family history of T2DM, and most of them were Chinese or Indian, employed full time, with a degree, and attending a private prenatal clinic. | Yes | No | Audio | No | Not reported | Reported at 15 interviews | No |

| Data analysis | | | | | Reporting | | | | | Reviewer |
|---|---|---|---|---|---|---|---|---|---|---|
| Number of data coders | Description of coding tree | Derivation of themes | Software | Participant checking | Quotations presented | Data and findings consistent | Clarity of major themes | Clarity of minor themes | Quality Assessment | |
| 2 | Yes | Themes derived from data | N/A | No | Yes in full quote tables by theme and sub theme | Yes | Clear | Clear | Medium | Small sample size, short interviews, however themes were clear and recruitment process was transparent |
| 2 | Yes | Themes derived from data | ATLAS.ti 6 | No | Yes in full quote tables by theme and sub theme | Yes | Clear | Clear | High | Despite small sample size, multiple interviews allowed for a richer dataset. |
| 1 but checked informally by members of research team | No | Themes derived from data and then refined based on existing evidence of topic | N/A | Yes | Some reported, no quote table | Yes | Clear | Clear | Medium | Small sample size, even though focus groups tend to be better with a smaller sample size, this was still quite low and does not allow for a rich data set and exploration of themes |
| 1 | No | Themes derived from data | N/A | No | Yes with a quote table | Yes | Clear | Clear | High | Good sample size, clear discussion of themes and recruitment |
| 2 | Yes | Themes derived from data | ATLAS.ti 6 | No | Yes in full quote tables by theme and sub theme | Yes | Clear | Clear | High | Despite small sample size, multiple interviews allowed for a richer dataset. |
| 1 | Yes | Themes derived from data | Not mentioned | Not mentioned - secondary analysis | No quote table, in text quotes included | Yes | Clear | Clear | Medium | A good sample size for a secondary analysis, but more information on primary data collection needed for transparency |
| 1 | Yes | Themes derived from data | Not mentioned | No | No quote table, in text quotes included | Yes | Clear | Clear | High | Good sample size and mixed qualitative methods, leading to an apparently rich dataset. Paper was clear. |
| 1 | Yes | Themes derived from data | Not mentioned | No | No quote table, in text quotes included | Yes | Clear | Clear | High | Good sample size , rich dataset and exploration of themes. Clear paper. |
| Not reported | No | Themes derived from data | Not mentioned | Yes participants were informed of core themes for member checking and saturation verification | No quote table, in text quotes included | Yes | Clear | Clear | Medium | Small sample size, but the discussion of themes was nuanced and rich. The interviews were also reported as being quite short. |
| Not reported | No | Themes derived from data | Not mentioned | No | No quote table, in text quotes included | Yes | Clear | Clear | Medium | Good sample size with rich, longer form interviews. Clear paper and recruitment process clearly explained. |
| Not reported | No | Themes derived from data | ATLAS.ti V.5 | No | No quote table, in text quotes included | Yes | Clear | Clear | Low | Very small sample size, potentially too low to make any assumptions about experiences of self-management |
| At least 2 | No | Themes derived from data | Nvivo 11 | No | Yes and quote tables included | Yes | Clear | Clear | High | Very large sample size, very rich dataset, clear paper and all processes communicated well |
| 2 | No | Themes derived from data | Not mentioned | No | Yes, embedded in article | Yes | Clear | Clear | High | Good sample size, clear paper and clear recruitment and analysis processes described |
| The lead researcher independently coded the data, and discussed the codes and themes with two experiences Kaupapa Māori researchers, who had access to the interview data | No | Themes derived from data | Not mentioned | Participants were invited to review, edit or change their transcript prior to data analysis. | Yes, embedded in article | Yes | Clear | Clear | Medium | Small sample size, but an exploratory study and acknowledged it was a sub set of women who are rarely interviewed about their experiences. |
| Each transcript reviewed and coded independently by two researchers | Yes | Themes derived from data | Not mentioned | No | Yes, embedded in article | Yes | Clear | Clear | High | Good sample size, clear description of analysis process and themes very clear and supplemented with quotes |
| 1 | Yes | Themes derived from data | Atlas Ti | No - but transcripts checked by an author for accuracy | Yes but quote tables not included | Yes | Clear | Clear | Medium | Based on the qualitative interviews alone the sample size is small. However, there was a rich discussion of themes and analysis was rigorous. |